

BRANCH PROFITABILITY EDUCATION SERIES

Why Your Census Number Is Not Arbitrary

What Nobody Tells Hospice Sales Reps About the Business — and the People — They Sell For

A Spartan Coaching Field Education Document · Branch Profitability Series

At some point early in your hospice sales career, someone gave you a number. Eight admissions a month. Twelve. An ADC of 60 by Q3. What it almost certainly did not come with was an explanation of where it came from — or what it actually means for the people your branch exists to serve.

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Before the Numbers

The People Behind the Number — Who Your Census Sustains

The census number you were given is not a business metric dressed up as a goal. It is a human requirement. Every patient admitted to your branch's service places an act of profound trust not just in an organization or a Medicare benefit — but in a specific team of specific people. That trust carries weight unlike almost any other in medicine: these are the people a patient and their family will invite into their home during the most vulnerable, most sacred chapter of a human life.

Your census — the number of patients on service at any given time — is what funds that team. It is what pays their salaries every two weeks, covers the medications in their nursing bags, puts fuel in their cars, and keeps the office running so they can focus entirely on the person in front of them. Without sufficient census, that team cannot exist in full. Without that team in full, the patients who trusted your branch do not receive the care they were promised.

"A patient does not just enroll in a benefit. They entrust a team of people with the most intimate chapter of their life — and their death. The census is what makes it possible to honor that trust."

Hospice is not a building patients go to. It is a team that comes to them — in their home, their assisted living room, their nursing facility bed — and wraps around them with expert, compassionate, coordinated care. The RN case manager who calls at 2 a.m. The hospice aide who helps a man bathe with dignity on what may be one of his last mornings. The social worker who sits with a daughter who has been managing medications, navigating insurance, and holding the family together for months — and finally tells her that she does not have to do this alone anymore. The chaplain who shows up without an agenda and simply asks what a person needs to say.

These are not support roles. These are the most influential people in the dying process. They shape what a patient's final weeks feel like. They determine whether a family member looks back on that time with peace or with regret. They are the reason hospice exists — and they are only there, in full, when the branch census is high enough to sustain them.

The number you were given is not a quota. It is the minimum condition under which the most influential people in hospice can actually do their jobs.

PART ONE

The Clinical Team: The Heart of Every Patient's Final Chapter

Every line item in a hospice payroll is a person. Not a position. Not a job title. A person with training, credentials, relationships, and the weight of walking into someone's home on one of the worst days of their family's life — and making it better. Understanding your branch's cost structure begins with understanding these people and what they actually do for the patients your census supports.

The Clinical Team

These roles are not optional. Medicare Conditions of Participation require every certified hospice to maintain this team regardless of census. They are hired, credentialed, trained, and retained — and their salaries are paid every two weeks whether the branch has 20 patients or 80. As census grows, the clinical team grows with it — by requirement, not by choice. A new RN Case Manager is added for every 12 patients. A Hospice Aide for every 8.

Clinical Role	What They Do for Each Patient	Annual Cost
Executive Director	Leads clinical operations, regulatory compliance, and team performance. Accountable for the standard of care every patient receives — and the first person responsible when that standard is not met.	\$140,000
Supervisor RN Case Manager	Manages the clinical staff and ensures care quality is consistent across every patient. Bridges the gap between the office and the bedside. Catches problems before they reach patients.	\$110,000
RN Case Manager (×2 min; +1 per 12 patients)	The primary clinical contact for each patient. Visits at home twice a week or more. Manages pain, assesses symptoms, adjusts medications, coordinates care, and — over weeks or months — becomes one of the most trusted people in a dying patient's life outside of their family.	\$100,000 each
Hospice Aide (×2 min; +1 per 8 patients)	Provides personal care — bathing, hygiene, comfort, and the preservation of human dignity. Often the team member with the most frequent contact. Provides hands-on care that keeps patients clean, comfortable, and connected to their sense of self in their final weeks.	\$50,000 each
Social Worker	Carries the weight that the family has been carrying alone. Navigates financial resources, insurance, facility communication, and family dynamics. Helps a daughter stop being her father's medical coordinator and start being his daughter again. Grief support, care planning, and the invisible work of holding families together.	\$75,000

Chaplain	Provides care for the dimension of dying that medicine cannot reach. Shows up without an agenda and sits with whatever a patient needs to say — fear, regret, faith, doubt, unfinished business, or silence. For many patients, the chaplain is the person they remember most.	\$70,000
After Hours RN	The voice on the other end of the 2 a.m. phone call. When a patient's breathing changes and their family panics, this is the person who answers, assesses, guides, and — when necessary — goes. No patient faces a nighttime crisis without a trained clinician available. This position makes that possible.	\$95,000
Weekend RN	Ensures care does not pause on Saturday and Sunday. A patient's need for pain management, symptom control, or clinical support does not observe business hours. This role ensures the clinical team is never truly off — because neither is the work.	\$95,000
Medical Director (Contract)	Certifies eligibility, oversees symptom management protocols, and provides physician-level oversight of every patient's plan of care. Required by Medicare for every certified hospice. The clinical authority who ensures each patient receives medically appropriate, evidence-based comfort care.	\$75,000

As census grows, so does the clinical team — by Medicare requirement, not by choice. An RN Case Manager is required for every 12 patients on service. A Hospice Aide for every 8. The branch cannot delay those hires without risking compliance — and, more importantly, without failing the patients already enrolled.

When census falls, these are the last positions eliminated — because eliminating them means abandoning patients.

What happens in practice when census drops and the branch cannot sustain this team fully? Leadership delays replacing a departing RN. After-hours coverage gets stretched thin. An aide carries more patients than she should. The chaplain's hours get cut. Training budgets disappear. The clinical team feels the pressure even if they cannot name its source — and the patients enrolled feel it too, even if no one says so out loud.

That is the human cost of insufficient census. It does not show up on a balance sheet. It shows up at the bedside.

The Operational Team — The Infrastructure Behind Every Visit

Behind every clinical visit is a referral processed, a schedule built, a supply ordered, a care plan documented, and a billing cycle completed. These roles exist so that the clinical team can remain focused entirely on patients — not paperwork.

Operational Role	What They Make Possible	Annual Cost
Intake Coordinator	Receives referrals, processes admissions, and coordinates the first clinical visit. Often the first human voice a patient's family hears after the physician says hospice. That first interaction — its warmth, its speed, its competence — shapes the family's trust in the branch before a single clinician walks through the door.	\$60,000
Secretary / Administrator	Scheduling, documentation, billing support, and office operations. The operational foundation that keeps the branch functioning — without which clinicians spend their time on paperwork instead of patients, and the entire care system slows down.	\$55,000

The Sales Rep — The Person Who Makes All of It Possible

Every role above depends on what you do in the field. The RN Case Manager only has a patient to care for because a referral was made. The social worker only has a family to support because someone had the right conversation with the right physician at the right time. The chaplain only has a place in the branch budget because census is high enough to sustain it.

Your Role	What You Actually Generate	Annual Cost
Sales Rep / Market Development Liaison	Referral relationships, admission volume, and census growth. The originating link in the entire chain of care. Every patient on service is there because a rep had a conversation, built a relationship, and followed through on a referral. Every position on this list — every person who walks into a patient's home — is funded by that work.	\$115,000

Your salary is a fixed cost paid whether you produce or not. But more than that — every salary on this list, every person who shows up at a patient's home, is funded by the census you build. You are not chasing a quota. You are funding a care team. You are the reason the chaplain has a job. You are the reason the after-hours RN answers at 2 a.m.

PART TWO

What It Costs to Care for Each Patient, Every Day

Beyond salaries, every patient enrolled in hospice generates a daily cost — the direct expense of delivering care to one person, one day at a time. These costs are variable: they scale with census, they vary by diagnosis and acuity, and they must be covered before a single dollar of payroll or profit is possible.

Daily Patient Cost	What It Covers	Per Day
Pharmacy / Medications	Pain management, comfort medications, anti-anxiety drugs, and symptom control. Standard acuity: \$22–\$25/day. Oncology or complex pain: \$44–\$80/day. The medication a patient receives is often the difference between a peaceful death and an agonizing one.	\$22–\$44
Durable Medical Equipment	Hospital beds, wheelchairs, oxygen concentrators, walkers, and commodes that allow a patient to receive care at home rather than in a facility. The equipment that makes home the right place to die.	\$10
Medical Supplies	Wound care supplies, nursing equipment, personal protective equipment, and clinical consumables used during home visits. High-acuity patients with complex wounds or multiple symptom needs push this significantly higher — \$20–\$30/day.	\$10–\$23
Clinical Travel	Mileage and drive-time cost for every home visit by RNs, aides, social workers, and chaplains. In rural territories, this can be significant. Every visit requires a clinician to travel — and travel costs money.	\$6
Other Patient Costs	Contracted therapy, interpreter services, volunteer coordination, bereavement services, and miscellaneous per-patient expenses that do not fit neatly into the categories above.	\$5
Total Daily Variable Cost	Per patient, per day — before salaries or overhead	\$53–\$88

High-acuity patients — those with advanced heart failure, ALS, end-stage COPD, or complex pain management needs — can push daily medication costs alone past \$80. These patients often have longer lengths of stay, which benefits the branch financially, but their daily care cost is significantly higher and must be factored into any honest financial model.

Monthly Overhead — The Cost of Having a Place to Do This Work

In addition to salaries and daily patient costs, every branch carries a layer of fixed operational overhead that exists before anyone walks through the door. Office rent, utilities, liability insurance, professional licensing fees, EMR

and billing software, compliance training programs, telephone systems, and administrative equipment — costs that have nothing to do with any specific patient but everything to do with being a functioning, licensed, compliant hospice.

Overhead Category	Monthly (Approx.)	Annual Total
Office Rent, Utilities, Insurance	\$18,000–\$22,000	\$216,000–\$264,000
EMR / Billing / Compliance Software	\$6,000–\$8,000	\$72,000–\$96,000
Training, Licensing, Credentialing	\$4,000–\$6,000	\$48,000–\$72,000
Administrative & Miscellaneous	\$8,000–\$12,000	\$96,000–\$144,000
Total Monthly Overhead	\$35,000–\$45,000	\$420,000–\$540,000 / year

Before a single salary is paid. Before a single medication reaches a patient. Before a nurse makes a single home visit. The branch is spending between \$420,000 and \$540,000 per year just to exist as a licensed, operating hospice. Census is what covers that cost — and everything above it.

Where the Revenue Comes From

Medicare reimburses hospice through a per diem model called Routine Home Care (RHC). For each day a patient is enrolled and receiving care, the branch receives a fixed daily payment from Medicare. This rate does not change based on how many visits were made or how much medication was used. It is a flat daily rate per patient — which is precisely why census is the primary driver of branch revenue. More patients on service means more days of reimbursement.

Medicare Payment Period	Daily RHC Rate (Approx.)	Why It Changes
Days 1–60 of a spell	\$208–\$224 per patient per day	Higher rate reflects the greater clinical intensity of the initial enrollment period — symptom management, family education, care plan establishment, and the significant front-loaded nursing and social work investment every new patient requires.
Days 61 and beyond	\$177–\$209 per patient per day	Rate adjusts downward after the initial high-intensity phase. Longer-stay patients are typically more stable and require less acute intervention. They continue generating per diem revenue while their care remains continuous and coordinated.

After subtracting daily patient costs (\$53–\$88), the remaining amount is the contribution margin — the portion of each patient day that goes toward covering salaries, overhead, and ultimately profit. That contribution margin is approximately \$120 to \$155 per patient, per day.

Every patient on census generates roughly \$120 to \$155 per day toward the fixed costs that keep your clinical team employed. Every vacant day — a patient who was never admitted, who left service early, who was never referred — is \$120 to \$155 that did not arrive. Multiply that by 365. Then multiply it by the number of patients between your current ADC and your break-even ADC.

PART THREE

The Numbers That Should Have Been Explained on Day One

Break-Even ADC — The Floor Below Which Everything Breaks Down

Break-even ADC is the Average Daily Census at which total revenue exactly equals total costs — salaries, daily patient costs, and overhead combined. It is not a goal. It is not a benchmark. It is the minimum viable condition for the branch to operate without losing money every single day.

$$\text{Break-Even ADC} = \text{Annual Fixed Costs} \div (\text{Contribution Margin per Day} \times 365)$$

A typical hospice branch requires between 35 and 55 patients on census at all times just to reach break-even. Below that threshold, the branch absorbs a daily operating loss. Those losses drain cash reserves, constrain hiring, and create financial pressure that eventually affects the quality of care — even if no one says so out loud.

What happens when a branch operates below break-even for an extended period? Leadership delays replacing a departing RN. After-hours coverage gets stretched thin. Supply orders get scrutinized before approval. Training budgets disappear. The clinical team feels the pressure even if they cannot name its source. And the patients — the patients who enrolled because they trusted this organization — receive care from a team that is operating under financial strain.

The human cost of a census that stays below break-even does not show up on a balance sheet. But it shows up at the bedside. In the RN who has one too many patients. In the aide whose hours were cut. In the chaplain who can only visit half as often. In the family that needed more support than the branch could give.

Target Margin ADC — The Number That Actually Funds the Mission

Break-even means the branch survived the month. Target Margin ADC means the branch is sustainable, investable, and able to grow. Healthy hospice organizations operate at an operating margin of 12 to 18 percent. That margin is not profit extracted from patients. It is the financial foundation that allows the organization to:

- Hire additional clinical staff before census demands require it, so new patients receive full attention from their first day of service.

- Invest in clinical training and continuing education that keeps the team sharp on pain management, symptom control, and family communication.
- Build administrative infrastructure that reduces errors and delays in care coordination.
- Expand into underserved communities where eligible patients are not yet receiving the care they deserve.
- Absorb market disruptions, regulatory changes, and reimbursement adjustments without cutting clinical positions.
- Create the conditions in which the most influential people in hospice can do their best work — without the weight of financial crisis in the background.

The difference between break-even ADC and target margin ADC is typically 15 to 30 additional patients on census. Those patients are not a rounding error. They are the difference between an organization that survives and one that thrives — and between a clinical team that is stretched and one that is fully supported.

Every admission above break-even is not just revenue. It is investment capital for a stronger clinical operation, a better-supported care team, and more patients who receive the expert, compassionate end-of-life care they were promised when they enrolled.

The Admissions Math — What You Actually Need to Produce

Census does not hold itself. Patients die. Some discharge. Some revoke. A branch must continuously admit new patients to replace those leaving service. The number of monthly admissions required is determined entirely by your target ADC and average length of stay.

$$\text{Monthly Admissions Needed} = (\text{Target ADC} \times 365) \div \text{Avg. Length of Stay} \div 12$$

Target ADC	Avg. Length of Stay	Monthly Admissions Needed	Weekly Needed
30 patients	60 days	~15 / month	~4 / week
50 patients	90 days	~20 / month	~5 / week
60 patients	90 days	~24 / month	~6 / week
80 patients	90 days	~32 / month	~8 / week
100 patients	90 days	~41 / month	~10 / week

If your branch targets 60 ADC with a 90-day average length of stay, it needs approximately 24 new admissions per month — 6 per week — just to hold census flat. Not grow it. Hold it. Two marketers splitting that production need to close 12 admissions each per month. That is not a soft goal. That is the math behind keeping every person on the clinical team employed and every enrolled patient cared for.

PART FOUR

Reading the Field Differently Now That You Know This

The conversations you have every day in the field look entirely different once you understand what is actually at stake behind the census number. Here is how to reread situations you have already experienced — now that you understand them.

When your manager says "we need three more admissions this month,"

They may not be talking about a quota. They may be talking about the break-even line. Three admissions at a 90-day average length of stay adds 7 to 8 patients to the running ADC over the following quarter. At \$135 per patient per day in contribution margin, that is roughly \$340,000 in annual revenue that either arrives or does not. The urgency is real. Now you know exactly why.

When a referral stalls and you do not follow up,

The cost is not a missed commission. It is a patient who did not get care — whose pain went unmanaged, whose family carried the burden alone, who faced the end of life without a team beside them. It is also a gap in census that the branch absorbs financially, paying every salary on that list while the revenue that would have covered it never arrived. Referrals do not stall themselves. They stall in the gap between your last contact and your next one.

When a patient is admitted with a very short prognosis,

A two-week patient barely covers their own admission cost after all direct expenses. The patients who sustain a branch financially — and who receive the most complete, meaningful hospice care — are those who were referred early, when the clinical team had months to provide comfort, education, and family support. Early referral is not just a financial win. It is the difference between a patient who had time to know their care team and one who met them for the first time days before dying.

When the clinical team seems stressed and understaffed,

There is likely a direct connection to census. Below break-even, the branch may be holding open positions, stretching existing staff thin, or delaying hires that the caseload genuinely requires. The RN who has one too many patients on her list is not a staffing failure — she is a symptom of a census that is not sustaining the model it needs. Your production is not separate from clinical quality. It is upstream of it. It is the reason clinical quality is possible at all.

When you understand that every conversation you have connects to the census,

The work becomes something different. Not a quota to survive. Not a metric to manage. A direct line between your effort in the field and whether a nurse shows up. Whether a chaplain can sit. Whether a family gets through the hardest days of their lives with expert, compassionate people beside them. You are not moving referrals. You are funding the team. You are the reason every person on that clinical list has a job — and the reason every patient who enrolls receives the full weight of what hospice is supposed to be.

The Branch Profitability Simulator

A Tool Built to Make This Visible

The Spartan Coaching Branch Profitability Simulator was built because this education almost never happens in the field. Sales reps are given goals without context. Leaders are given dashboards without explanations. The connection between daily census, clinical staffing, patient costs, and care quality remains invisible to the people whose work most directly determines it.

Tool Feature	What It Gives You
Break-Even ADC	Enter your actual revenue rates, clinical variable costs, and staffing assumptions to calculate the precise minimum census that covers all expenses — the floor below which the branch loses money every day.
Target Margin ADC	Set your operating margin goal (12–18% is standard for a healthy branch) and see exactly what census is required to achieve it. This is your real sales target — not just survival, but the census that builds a sustainable, investable organization.
Full Clinical Staffing Model	See how clinical payroll scales as census grows, including which roles are triggered at which patient thresholds (an RN at every 12 patients, an aide at every 8). Understand exactly which people your census sustains — and which ones disappear when it falls.
Daily Patient Costs by Acuity	Model standard, blended, and high-acuity patient populations to understand how diagnosis mix (oncology vs. cardiac vs. COPD) affects variable costs, contribution margin, and overall financial performance.
Monthly Admissions Required	Convert your target ADC into the exact monthly admission number your team must produce — based on your actual average length of stay. Know the number. Know what it means. Build your sales plan around real math.
Marketer Productivity Targets	See how many admissions each marketer needs to produce to sustain the census goal. Understand what your individual monthly production target means in the context of the full branch plan — with the math visible, not assumed.
Cash Runway Projection	Model how long starting capital can sustain a branch operating below break-even while census builds. Know which month cash flow turns positive. Determine whether your ramp plan is realistic before you commit to it.
Profitability Curve	Visualize how annual profit and operating margin change across the full census range — from 10 to 200 patients. See break-even, target margin, and your current ADC on the same chart. Understand where you are and exactly how far you have to go.

You have never had access to this information in a form you could use in the field. The simulator puts the complete financial picture of your branch in front of you — not as a report to review after the fact, but as a working model you can adjust, test, and use to understand the real stakes of every admission you close or lose.

Access the Branch Profitability Simulator at spartancoaching.com

What This All Means

The Census Is Not the Point. The Care Is.

Everything in this document — the staffing model, the daily costs, the break-even math, the revenue rates, the target margin — exists to support a single reality: somewhere in your territory, right now, there is a person who qualifies for hospice care and is not receiving it.

Their family is managing their medications without guidance. Their pain is being managed inadequately or not at all. They are facing the end of life without a team of professionals who know their name and their wishes — without an RN who has visited them enough times to understand their disease trajectory, without a social worker who knows what their family needs, without a chaplain who has built the kind of trust that takes time.

That gap is not clinical. Physicians understand hospice. Nurses understand hospice. The gap is conversational. A referral that was not made because the rep did not have the right language. A physician who said 'not yet' because no one responded with clinical confidence. A patient who was never asked because the rep's calendar was full of low-value visits to accounts that were never going to convert.

"The census is not the point. The census is what makes the point possible. The point is that a nurse shows up. A social worker calls. A chaplain sits. A family gets through the hardest days of their lives with people beside them who know exactly what they are doing and why they are there."

Your work in the field is the beginning of that chain. Every conversation you prepare for. Every referral you follow through on. Every physician you educate, every discharge planner you support, every family you help get to the right decision at the right time — that work funds the clinical team that carries it forward. That work is the reason every person on that staffing list has a job. And that work is the reason every patient who enrolls receives the full weight of what hospice is supposed to be.

When you understand that, the number is not arbitrary. It never was. You just needed someone to explain it.

Key Terms — Quick Reference Glossary

Term	Definition
Average Daily Census (ADC)	The average number of patients on hospice service on any given day. The primary revenue driver for every branch — each additional patient generates \$120–\$155 per day in contribution margin toward fixed costs, payroll, and ultimately the ability to sustain the clinical team.
Break-Even ADC	The minimum census at which total revenue exactly covers all costs — salaries, patient costs, and overhead. Below this number, the branch loses money every single day. Typically 35–55 patients for a standard community-based branch. This is the floor below which the clinical team cannot be fully sustained.
Target Margin ADC	The census needed to achieve the operating margin goal (typically 12–18%). This is the real sales target — not just survival but sustainability. The gap between break-even and target margin ADC is usually 15–30 patients, and it represents the difference between an organization that survives and one that thrives.
Contribution Margin	Revenue per patient day minus direct patient costs. Approximately \$120–\$155 per patient per day in a standard acuity branch. This is the amount each enrolled patient generates toward the fixed costs that keep the clinical team employed and the branch operational.
Routine Home Care (RHC)	The Medicare per diem rate paid for each day a patient is enrolled in hospice. Days 1–60 are reimbursed at a higher rate than days 61 and beyond. The primary revenue mechanism for every hospice branch — a flat daily payment for each patient on service.
Monthly Admissions Needed	New patient enrollments required each month to maintain target ADC, calculated as $(\text{Target ADC} \times 365) \div \text{Avg. LOS} \div 12$. This number directly drives marketer headcount, individual production targets, and the entire sales plan.
Cash Runway	How many months starting capital can sustain a branch operating below break-even before reserves are exhausted. Critical for new branch launch planning — the ramp timeline must reach break-even before runway runs out, or the branch will not survive long enough to fulfill its mission.
Average Length of Stay (LOS)	Average number of days a patient remains on service before death or discharge. Directly determines monthly admissions volume: shorter LOS means more admissions are needed to hold census. Longer LOS shifts revenue toward the lower Day 61+ rate but reduces the monthly admission requirement.
Operating Margin	Annual profit as a percentage of annual revenue. A healthy hospice branch targets 12–18%. This margin is not extracted from patients — it is the financial cushion that allows the branch to invest in clinical staff, expand to underserved markets, and sustain the care team through disruptions.

Access the Branch Profitability Simulator and full field education library at spartancoaching.com

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Every input is customizable. Every output is actionable. Start with your real numbers.